

St. Clair Hospital

1000 Bower Hill Rd, Pittsburgh, PA 15243

DISCHARGE SUMMARY Document #: DIS-008

Patient & Admission

Name	Tamara Higgins
Date of Birth	1970-10-04
Phone	412-555-5947
Address	374 Jackson Rest Greensburg, PA, 15601
MRN	MRN-496631
Admit Date	2026-04-12
Discharge Date	2026-04-19
Attending	Ryan Thompson, MD
Attending NPI	5787133151

Insurance

Primary Payer	Cigna
Primary Member #	U531003309

Primary Discharge Diagnosis

NSTEMI, mid-LAD distribution

ICD-10: I21.4, I25.10

Hospital Course

The patient presented to the emergency department with acute onset substernal chest pain and dyspnea. Initial electrocardiogram demonstrated ST-segment depression in the anterior leads with reciprocal changes. Troponin I was elevated at 2.8 ng/mL, consistent with acute myocardial infarction. Coronary angiography revealed a critical stenosis in the mid-left anterior descending artery with TIMI 2 flow. The patient underwent successful percutaneous coronary intervention with placement of a drug-eluting stent to the LAD with restoration of TIMI 3 flow. Transthoracic echocardiography performed on hospital day two showed a left ventricular ejection fraction of 38% with anterior wall hypokinesis and mild mitral regurgitation. B-type natriuretic peptide was mildly elevated at 285 pg/mL.

During the hospital course, the patient was initiated on dual antiplatelet therapy with aspirin and ticagrelor, along with high-intensity statin therapy. An ACE inhibitor and beta-blocker were uptitrated as tolerated for cardioprotection and heart failure management. The patient remained hemodynamically stable throughout the admission without recurrent chest pain or arrhythmia. Cardiac rehabilitation was consulted and the patient participated in early mobilization with good tolerance. Repeat troponin levels trended appropriately downward, and the patient was discharged on hospital day seven in stable condition.

The patient is being discharged on optimal medical therapy with close outpatient cardiology follow-up scheduled within two weeks. Repeat transthoracic echocardiography is planned in six weeks to reassess left ventricular function and guide further therapeutic decisions. The patient has been counseled on medication adherence, dietary sodium restriction, and participation in phase II cardiac rehabilitation. He was advised to seek immediate medical attention for recurrent chest pain, shortness of breath, or syncope.

Procedures Performed

Coronary angiography	CPT 93458
PCI to LAD with DES	CPT 92928

Medication Changes

Metoprolol succinate 50 mg daily	STARTED
Clopidogrel 75 mg daily	STARTED

Follow-up Instructions

Recommended Specialist	Cardiology
Follow-up Window	6 days
Urgency Tier	MEDIUM
Clinical Flags	post-acute-MI

Dictated and electronically signed by:
Ryan Thompson, MD
NPI: 5787133151
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